

--

Name: إيمان عاطف محمد محمود National ID: 27906042100081	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



STRENGTHENING THE

٨ / ٨١ حتى ١٨٠٠ - المجزء

YV9.7.2 Y1.00.11

張



حاجتہ علی جاوید پوری فنونِ تصنیف



F

الْبَطْنَةُ سَارِيَّةٌ حَتَّى ٢٠٣١/٠٨/١٤





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : اسم المريض محمد محمود

Sex:

3- Medical number:

Female

83270903

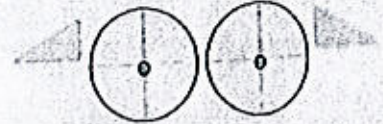
Old Med:

Old Med

1137-2025

2-ID number: 27906042100081

Patient Age : 46 Y



Family history

Family history
+ve

Phone

Phone
01119436861

BMI

BMI

REASON OF MDT

REASON OF MDT
N/A

marital status

Marital Status
M/S

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2025-04-27

☐

Co morbidity

Co Morbidity
htn

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

pet ct 3/2025:lt breast UOQ lesion
multiple bilobar HFL
multiple lytic bony vertebral lesion

BX

IDC

BX COMMENT

ER

8/8

ER COMMENT

PR

4/8

PR COMMENT

Her 2 Neu

3

Her 2 Neu COMMENT

SISH

SISH COMMENT

KI67

>20

T

N

M

1) BONE ,
LIVER

Sonomammography

PATIENT HISTORY

denovo MBC to liver , bone
diagnosed 11/2024

received 6 cycle TC last 7/4/2025+2 cycle hecreptin SC last 3/2/2025+received zometa twice last
14/4/2025-->good clinical response subjectively+no peripheral neuropathy

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR TAXOTER / HERCEPTIN FOR MORE 3 CYCLES THEN REEVALUATE
ZOMETA / 3 MONTHS

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/مائل حاتم د/إبراهيم خلد	RADIOLOGY	RADIOLOGY د/أحمد محمد
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: ايمان عاطف محمد		
MR. : 879		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
46 YRS OLD FEMALE PT LT breast UOQ lesion, multiple bilobar HFLS, multiple lytic bony vertebral lesions. MBC to liver and bone (denovo) Recommendations-> Dx in 11/2024 as denovo MBC to bone and liver Recieved 6 cycles T.C q.3 weeks, last on 7/4/2025 +2 cycles s.c hercptin, last 3/2/2025 +recieved zometa twice, last on 14/4/2025 at Shiekh zayed hospital with good clinical reponse+ no peripheral neuropathy Referred to continue ttt at Herme Bx-> tru cut LT IDC GII in 11/2024 ER: 8, PR: 4, Her 2: +3, Ki67:72 MDT 10/8/2025 DECISION For maintenance Herceptin + AI + follow up E2 leve FOR FEMARA + SUPP HERCEPTIN IV PENDING REPORT ECHO >> R prem left breast cancer ct4 n1 m1 idc 2 er8/8 pr4/8 her23= met to liver bone rec 6 taxotere endoxan then 3 taxotere with herceptin st ai from 8/2025 cts 7/2025 no hfl cts 16/3/2026 d1 sclerotic lesion no hfl sm 4/2026 partial radiological response bs 4/2026 sd ef 62% 24/5 e2 fsh postm for herceptin 440 and femara cts only tiny lytic and sclerotic foci in iliac and vertebral lesions fpor comp for herceptin 440 and femara CTs with comparison .. SD, for herceptin + femara , for echo next time, + zometa next month CTs with comparison .. SD for herceptin + femara 9/2026 echo :65%		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
	الطبيب المعالج : نهى سعيد علي محمد	
يعتمد ,		



Report Number: m2272

Female	: النوع	ايمن عاطف محمد محمود	: اسم المريض
الشيخ زايد	: العنوان	44 Y	: السن
12/11/2024 11:25	: تاريخ استلام العينة		: تاريخ اخذ العينة
18/11/2024 10:44	: تاريخ التقرير	مستشفى الشيخ زايد التخصصي	: طالب التحليل
		مستشفى الشيخ زايد التخصصي	: عناية

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by: -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / XT .

Sample recived was treated by :- CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: strong nuclear staining of > 60 % of tumour cells (core biopsy)

PR: weak nuclear staining of 30 % of tumour cells (core biopsy)

HER2:neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells (core biopsy) .

Ki67 :positive nuclear staining of > 20 % of tumour cells (core biopsy)

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 8 / 9)

PR: POSITIVE (SCORE 4 / 8)

HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3+)

#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS > 20 % .

Pathologist:

د. نورا ماهر

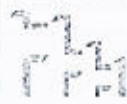
Lab Director:

د. نورا ماهر

19 El-sheikh Rehan Street, Abeideen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website http://www.healthlabs.net
email: pathologychl@yahoo.com
facebook: قسم الباثولوجي - الإدارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها



Name	ايمن عاطف محمد	Age	44y	Code	893-24
Referred by		Date of receiving specime	7/11/2024		
Patient ID		Sex	Female		
Clinical data	Left breast mass				
Specimen	US guided biopsy				

PATHOLOGY REPORT

Gross picture: Core biopsy specimen, totally embedded.

Microscopic Picture: Sections examined reveal tissue cores infiltrated by malignant cells arranged in clusters with moderate tendency to form tubules. The neoplastic cells show moderate degrees of anaplasia. No in-situ component seen in this biopsy.

Diagnosis: *Left breast mass lesion; US guided biopsy:*

▪ Invasive duct carcinoma, Grade II.

Prof. Dr. Mohamed Sherif, MD

Patient Name: Eman Atef Mohamed Mahmoud Mohamed	Date : 01/12/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID: 1137/2025	Pat. ID: 100436

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

A case of breast cancer , compared to last report dated 10-7-2025, the current study revealed

I Chest:-

- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Prominent superior mediastinal soft tissue mostly thymic in nature.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous fatty CT texture. With right lobe hyperdense wedge shaped area at segment VII likely focal fat sparing
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton: still noted** multiple lytic lesions are seen at right iliac bone , S1 , L5 and L3 vertebrae.

Opinion:

- Stationary course since the last study.

Name : Iman Atif Mohammed Mahmoud
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 10-11-2025 11:27
Medical No. : 245892
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 46-year-old lady, with history of metastatic breast cancer.
- The prior PET/CT study in July 2025 was reviewed.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Foci of enhanced tracer uptake at right parietal bone, multiple bilateral ribs, multiple dorso-lumbar vertebrae, both iliac bones and proximal both femora.
- No other abnormal active or cold focal bony lesions detected.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Multiple metastatic osseous lesions as described.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient's Name: Iman Atif Mohammed Mahmoud
Date: 2026-07-18
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA. no available previous DICOM images for comparison.

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Small hiatus hernia.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing diffuse low parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Cholecystectomy clips are noted.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. Tiny right renal stone. No back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few scattered tiny vertebral and right iliac bone lytic and sclerotic foci with D1 vertebral body wedging.

Opinion:

❖ **No pulmonary or hepatic metastatic lesions.**

❖ **Few tiny scattered vertebral and right iliac bone lytic and sclerotic foci with D1 vertebral wedging for bone scan.**

Dr.sherif Wagdy

Name : Iman Atif Mohammed Mahmoud

Date : 17-09-2026 11:42

Branch : Dokki

Medical No. : 463856

Ref.Physician Prof: Goustav Roussey Egypt

Ins.Company : تعافق مستشفىفات

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 47-year-old female with left breast carcinoma. History of osseous metastases. Referred for follow-up skeletal evaluation compared to prior study dated April 6, 2026.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Marked reduction is noted in both the intensity and extent of radiotracer uptake across previously documented active metastatic sites, including the skull, sternum, shoulder girdles, pelvis, and bilateral femora.
- Residual focal uptake is limited to the L2 vertebra, the lowermost left rib (lateral arc), and the mid-lower right ribs laterally.
- No new areas of abnormal osteoblastic uptake are identified.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Significant scintigraphic improvement with marked partial response compared to the prior study dated April 6, 2026, reflecting favorable response to ongoing targeted systemic therapy.
- Residual active foci are restricted to L2 and bilateral lateral rib arcs.
- No new osteoblastic skeletal metastases detected.

Dr. Alaa Nagi

MD Nuclear Medicine

ECHO REPORT

NAME: 3 Cpl L E

DATE: 6 / 9 / 2026

ID NUMBER: /

SEX: / AGE: /

EF	<u>65%</u>	55-75%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.5</u>	3.5-5.8cm	AO.R		
LVEDS	<u>2.3</u>	2-4cm	LA	<u>2.3</u>	3-3.7cm
FS	<u>30%</u>	25-45%	RV	<u>3.6</u>	2-4cm
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		0.7-2.3cm(supine)
					1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function Normal

SWMA Normal

LA and Ao Root Dimensions Normal

Mitral Valve Normal

Aortic Valve Normal

Tricuspid Valve Normal

Pulmonary Valve Normal

Right Side and Systolic Function Normal

Intracardiac Masses and Effusion Normal

DR:

[Signature]